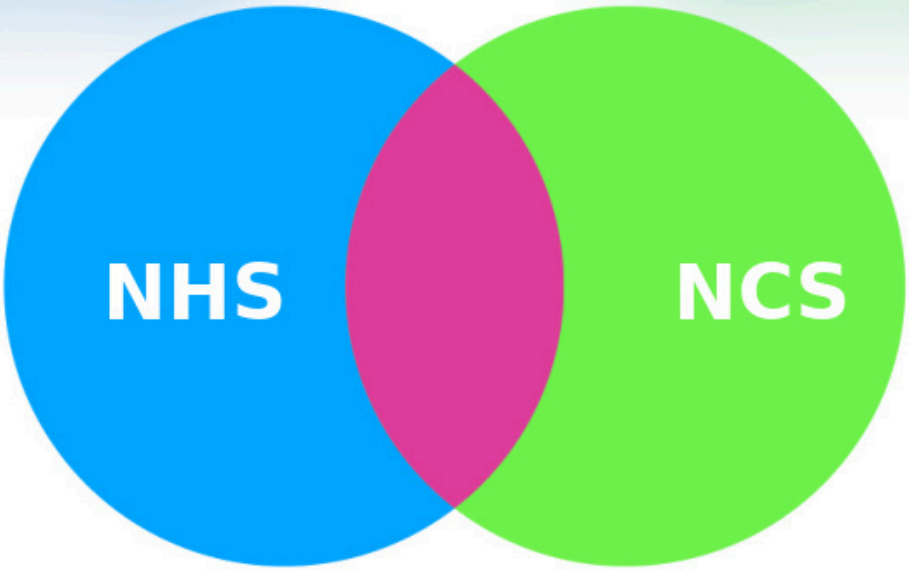
Connecting the Dots

Paving the way towards our National Care Service



Our pilot tests whether connected support helps carers and the people they care for return to work sooner, improves outcomes and generates cost evidence to help fund a future connected National Care Service.

The problem we are solving. By connecting the dots.

Carers are left to connect the dots.

Government agencies often work in separate systems. Information and responsibility do not reliably travel with the person, so families are left to repeat their story, chase updates and coordinate handovers themselves.

49%

of 14+ day delayed discharges were linked to social care capacity, transfer delays or access to rehabilitation / recovery.

26.8%

of surveyed unpaid carers found it fairly or very difficult to find information or advice about support, services or benefits.

20.9%

of surveyed unpaid carers said they had no encouragement or support in their caring role.

Sources: CQC / NHS England, March 2025; DHSC Survey of Adult Carers in England 2025-26. See evidence pages for definitions and scope.

WHAT THE PILOT TESTS

Connect the handovers, not just the services.

In five locations, the pilot will test whether named handovers and a shared picture of each family’s needs reduce the coordination burden. The time carers currently spend navigating the system can instead go to their own lives, wellbeing, learning, training or work - and to the person they care for.

Connecting the Dots

CASEY COMMISSION / PUBLIC REVIEW EDITION / OCTOBER 2026

How to Fund Our National **Care Service.**

Connecting the Dots

Two national services. One connected life. A co-produced proposition to connect health, care and opportunity around people, and test the economic value of better outcomes.

Prepared by Aubrey Ghose

Unpaid carer and Richmond & Wandsworth co-production contributor

PILOT FIRST. PLATFORM SECOND. EVIDENCE BEFORE SCALE.

TWO NATIONAL SERVICES.
ONE CONNECTED LIFE.



National Health Service

Your health. Connected.

National Care Service

Your care. Connected.

The Government has committed to establishing a National Care Service. The Casey Commission has been asked to help determine how it should be implemented.

The opportunity is to build care as a connected national service - able to work alongside a connected health service.

CONNECT CARE.

RELEASE CAPACITY.

REDUCE COST.

IMPROVE OUTCOMES.

CONNECTING THE DOTS

A co-produced proposition for the Independent Commission into Adult Social Care

A National Care Service **alongside the NHS**

Two national services, working together around the same household.

National Health Service
Your health. Connected.
High-quality healthcare, free at the point of use, for everyone in England.

National Care Service
Your care. Connected.
One recognisable front door connecting the public services that surround care — around the person who needs it and the people who care for them.

WHAT CARE NEEDS TO CONNECT

- **Adult Social Care** — Support, assessment and care services
- **Housing** — Safe, suitable and stable homes
- **DWP & Benefits** — Income, entitlements and financial support
- **Recovery & Rehabilitation** — Treatment, therapy and long-term support
- **Mental Health** — Integrated physical and mental health support
- **Learning & Training** — Skills, confidence and personal development
- **Employment** — Support into, in and back to work
- **Community Support** — Voluntary, community and peer support

ONE HOUSEHOLD. TWO PEOPLE. ONE PLAN. A MORE CONNECTED LIFE.

Connect services and decisions around the household, with consent and clear responsibility.

One household. **One shared 360° view.**

A shared, permissioned view of each care relationship: the household where relevant, never a merged identity.



ORGANISE THE PUBLIC SERVICES WE ALREADY HAVE AROUND ONE GOAL

SEE

Both voices and relevant facts

ACT

One lead and explicit next steps

MEASURE

Independence, time, work and cost

THE NATIONAL CARE SERVICE STARTS WITH A SHARED WAY OF WORKING

The switch is operational, not a claim that IT can be connected overnight: consent, lawful access, shared goals, named owners and measured outcomes. Each agency keeps its legal duties and existing systems.

What the evidence **is built on**

Carers, carees, people with lived experience and professionals contributed across a documented process.

11
meeting dates

9
official records

338
source-linked entries

41
named participants

16
directly attributed

6
recurring findings

WHAT PEOPLE DESCRIBED

Repeating information, chasing handovers and coordinating support across services while trying to care.

A documented pattern, gathered over three years of co-production.

The recurring burden is not a lack of effort by carers. It is the work of joining up support that arrives in separate pieces.

03 / THE FINDINGS

Six recurring findings.

Practical changes identified across the Richmond and Wandsworth co-production record.

01 **Joined-up services + shared responsibility**

Make hand-offs and ownership visible across organisational boundaries.

02 **One route to accurate information + records**

Create a reliable route to the right service so carers do not reconstruct the story.

03 **Earlier carer support**

Identify carers early; plan assessment, respite and contingency.

04 **Prevention + continuity before crisis**

Keep support connected through discharge, accommodation and changes in responsibility.

05 **Accessible communication**

Connect people to information while keeping a clear human route and alternatives.

06 **Co-production that closes the loop**

Show what changed because people contributed: action, owner, date and outcome.

PROVENANCE & APPROVAL

Source: Richmond Borough and Wandsworth co-production records. Findings are offered for working-group confirmation, not as unanimous endorsement; provenance to be checked before approval.

04 | NATIONAL CORROBORATION

The same six issues appear **in national evidence.**

01 | JOINED-UP SERVICES + SHARED RESPONSIBILITY

49%

Of delayed discharges after acute stays of 14+ days were linked to social care capacity or transfer delays (23%), or access to rehabilitation, reablement or recovery (26%).

Source: CQC, The state of health care and adult social care in England 2024/25; NHS England data, March 2025.

02 | ONE ROUTE TO ACCURATE INFORMATION + RECORDS

26.8%

Of surveyed unpaid carers said it was fairly or very difficult to find information or advice about support, services or benefits.

Source: DHSC, Survey of Adult Carers in England 2025-26, information and advice.

03 | EARLIER CARER SUPPORT

20.9%

Of surveyed unpaid carers said they had no encouragement or support in their caring role.

Source: DHSC, Survey of Adult Carers in England 2025-26, support and services.

Scope: SACE measures carers' experience of support information; it does not measure shared records or repeated storytelling.

05 | NATIONAL CORROBORATION

The same six issues appear **in national evidence.**

04 | PREVENTION + CONTINUITY BEFORE CRISIS

335k

New-client requests for adult social care support in 2024-25 originated at hospital discharge: 17% of all such requests.

Source: DHSC, Adult social care activity report, England 2024-25, route of access.

05 | ACCESSIBLE COMMUNICATION

12.1%

Of adult social care service users who tried to find information or advice found it very difficult; 46,760 people in the survey.

Source: DHSC, Adult Social Care Survey 2024-25, knowledge and information.

06 | CO-PRODUCTION THAT CLOSES THE LOOP

38.1%

Of carers who knew discussions about the cared-for person's services had happened said they were always involved or consulted.

Source: DHSC, Survey of Adult Carers in England 2025-26, involvement in arranging services.

Scope: these indicators corroborate the themes; they do not reproduce the local meeting sample or measure whether feedback changed services.

Health and care: **one system.**

The NHS feels the cost of a broken handover.

REASONS FOR 14+ DAY DELAYED DISCHARGES | ENGLAND | MARCH 2025

49%

Nearly half linked to social care capacity, transfers, rehabilitation or recovery.

23% + 26% in CQC's analysis of NHS England data.

23%

Lack of social care capacity or delays completing transfers to social care.

26%

Delays accessing rehabilitation, reablement or recovery services.

CASEY'S REMIT RECOGNISES THE HEALTH–CARE INTERFACE

Its terms of reference call for a data-driven review of NHS and council funding at the health–care interface, with reforms to reduce unnecessary hospital admissions and delayed discharges. Connecting the Dots offers a small local test of shared oversight and named handovers: the same system question, measured in practice in Richmond and Wandsworth.

Sources: CQC, The state of health care and adult social care in England 2024/25, reporting NHS England discharge reasons for March 2025 (acute stays of 14+ days); Casey Commission terms of reference, GOV.UK, updated 1 October 2026.

Millions of people.

One household at a time.

The scale is national, but improvement starts with a practical relationship and a measurable result.

5.8m

UNPAID CARERS | UK

Most recent comparable UK census total (2021). Many already work or are retired.

1.1m

LEFT PAID WORK TO CARE

Estimated number who left jobs to care in the two years before September 2026; polling, not a census.

889k

LONG-TERM CARE | ENGLAND

People receiving council-arranged long-term support during 2024-25; published figures subject to planned corrections.

16.7m

DISABLED PEOPLE | UK

2024-25 estimate. This is context, not a count of people needing care or suitable for paid work.

£29.4 BILLION

Gross local authority adult social care expenditure in England, 2024-25. This is existing spending - not a sum the pilot can simply redirect.

These populations overlap. They must not be added together or treated as a ready-to-work workforce.

Sources: Carers UK, DWP Family Resources Survey and DHSC 2024-25 (links in evidence note).

Before we connect the dots, **count them.**

The NHS has stronger internal pathways for its own services. The wider support system crosses organisations with different owners, duties and budgets.

INTEGRATED CARE SYSTEM (ICS)

A local partnership: NHS organisations, councils and other community partners plan together.

NHS SERVICE ARCHITECTURE

More connected internally — with seams between providers.

ICB | local NHS planning and commissioning

- **Primary care** — GP practices and PCNs
- **Hospital & specialist** — Trusts and specialist services
- **Community health** — Community teams and services
- **Mental health** — NHS mental health services

WIDER SUPPORT

Distinct owners and routes

- **COUNCIL** — Adult social care
- **HOUSING** — Council / housing providers
- **DWP** — Benefits and employment support

PARTNER BOUNDARY

ONE HOUSEHOLD | TWO PEOPLE: CARER + CAREE

Shared home, shared care: both carer and caree belong in the support picture.

TWO PEOPLE. TWO SETS OF RIGHTS.

A shared care relationship must never erase either person's needs or identity. Relevant decisions, support and accountability should be visible, with consent and each person's rights respected.

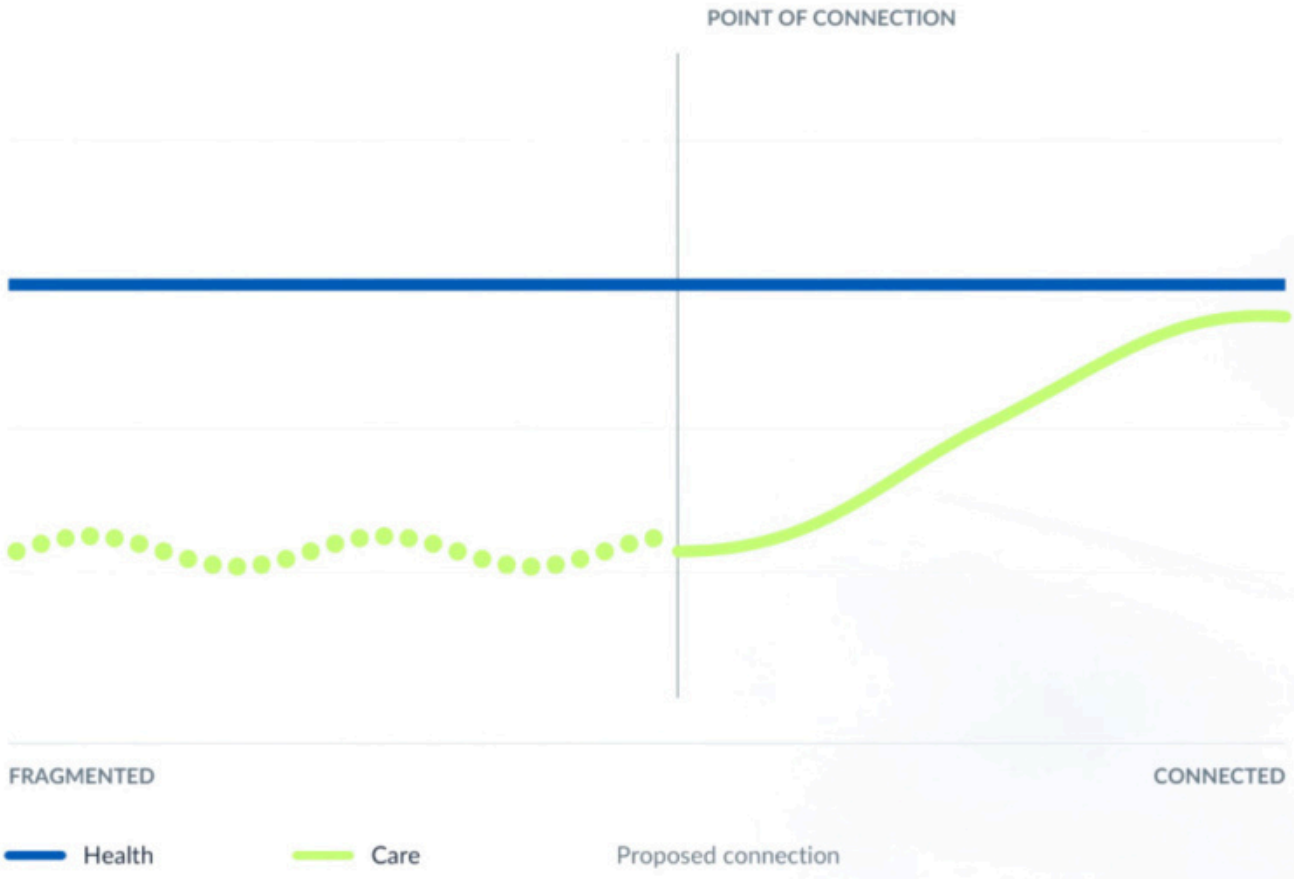
CONNECTION IS ALSO A HUMAN EXPERIENCE.

The test will ask carers and people receiving support whether involvement is respectful, accessible and useful - and whether it leads to better decisions.

04B / THE SYSTEM BENCHMARK

Two services.
One disconnected experience.

The NHS is the national reference. Care and wider support are fragmented across organisations. The proposed method connects decisions, not databases.



The green dots illustrate disconnected care encounters, not a measured time series. Their transition into one pathway is the proposed bridge towards care working alongside the NHS. The six local findings and national evidence describe the barriers this pilot would address.

CONCEPTUAL MODEL / NOT MEASURED RESULTS OR A FORECAST

A 100-family pilot across five locations.

20 families in each location, including Richmond & Wandsworth as one borough.

100
families in total

5
pilot locations

20
families per location



Five pilot locations

- **Newcastle** — 20 families
- **Manchester** — 20 families
- **Birmingham** — 20 families
- **Bristol** — 20 families
- **Richmond & Wandsworth** — 20 families

How it will work

01 Connect relevant services
02 Name the handovers
03 Measure before and after

WHAT THE SPREAD ADDS

Testing in five places creates the opportunity to learn across a wider mix of communities, demographics and local service settings. Equal groups make it easier to compare delivery, see what works consistently and identify where local adaptation is needed. This is a feasibility and early-outcomes pilot, not a nationally representative sample.

A LEARNING SYSTEM FOR THE PILOT

Learn across services, in real time.

A light-touch, AI-enabled workflow could help each local team connect agreed information, notice patterns and improve handovers across its 20 participating families.

01 Capture

Families and staff agree a shared goal plan and what information can be used.

02 Connect

A simple local workflow supports named handovers and flags gaps for staff to review.

03 Learn

An AI-supported view groups feedback and outcomes to reveal recurring barriers and patterns.

BACK-OF-HOUSE DASHBOARD

Patterns, progress and staff feedback - together.

Show progress towards family goals, recurring handover gaps, service feedback and emerging outcomes across locations. Staff review the signals; the system supports coordination and learning, not care decisions.

At six months, review efficacy, delivery cost, family and staff experience, and whether to continue or expand the test.

Carers **Connect.**

One connected front door to health, care, home and work.



ONE HOUSEHOLD. TWO PEOPLE. ONE PLAN. A MORE CONNECTED LIFE.

A practical service concept: the right information, the right handover and the right support, in one everyday place.

CONCEPT VISUAL - a proposed service, not a current government application

Start with achievable gains.

Then tackle harder barriers.

All households receive support from the outset; the stages describe what outcomes we can test and learn from first.

01

EARLY GAINS

Keep or restore paid hours

Help willing carers resolve missed information, repeated assessments, coordination delays and barriers to flexible work.

MEASURE: Hours of paid work retained or regained; time saved.

02

NEXT OPPORTUNITIES

Learning, training and supported work

Coordinate rehabilitation, transport, benefits advice, employment support and accessible training where the person wants them.

MEASURE: New participation, confidence, skills and work readiness.

03

COMPLEX NEEDS

Stability before employability

For families facing multiple risks or high care needs, join decisions across health, housing and support; do not make employment a condition of care.

MEASURE: Continuity, avoided crises, independence and wellbeing.

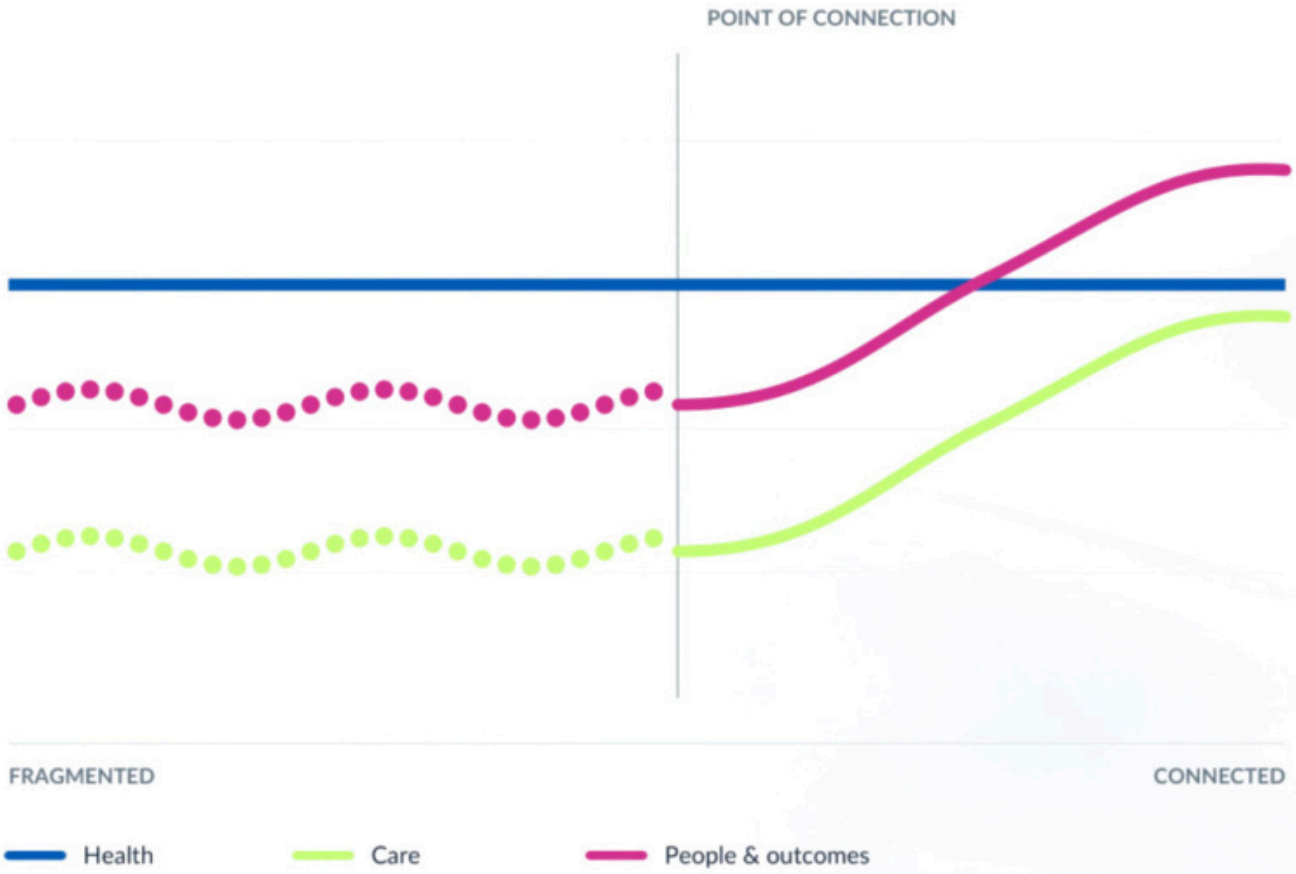
20 CONSENTING HOUSEHOLDS. 10 + 10. SIX MONTHS.

Baseline first. Track change in every household. Record harms and non-improvements. Independent review.

07C / THE OUTCOMES HYPOTHESIS

Connect care.
Improve lives.

The third pathway makes the test visible: when health and care decisions connect, can recovery, independence, learning and work improve?



Blue represents the NHS reference; green, coordinated care; fuchsia, the experience of carers and people receiving support. The two rising pathways use the same geometry to illustrate a hypothesis: better coordination may create the conditions for improved recovery, independence, learning and work.

THE PILOT TESTS WHAT CHANGES, FOR WHOM, AT WHAT COST.

CONCEPTUAL MODEL / NOT MEASURED RESULTS OR A FORECAST

07B / THE ECONOMIC MECHANISM

Recovery first.
Then measure the return.

A transparent scenario, not a forecast: value restored paid work, then separately test any fiscal effect.

ONE WILLING CARER REGAINS PART-TIME WORK

8 hours/week × £13/hour × 48 weeks

£4,992

GROSS ANNUAL EARNINGS RESTORED

1 PERSON	1,000 PEOPLE	58,000 PEOPLE
£4,992	**£5.0m**	**£289.5m**
illustrative annual earnings	illustrative annual earnings	illustrative annual earnings

THE PUBLIC FINANCE TEST

ADDITIONAL TAX
Verified tax receipts

BENEFITS
Changes in expenditure

SERVICE USE
Avoided repeat or crisis work

COST TO DELIVER
Staff, systems and evaluation

EARNINGS ARE NOT CASHABLE PUBLIC SAVINGS.

Net fiscal gains must be demonstrated against additional costs. The 1% scenario is arithmetic, not a forecast.

Help us test, measure **and build the evidence.**

This demonstrator is a practical way to test whether connecting care services around people improves the experience and use of public resources.

This proposition is offered in the context of the Government's commitment to a National Care Service and the work of the Independent Commission on Adult Social Care, chaired by Baroness Louise Casey of Blackstock DBE CB.

CONNECT services – **SAVE** time – **RELEASE** capacity – **IMPROVE** recovery – **SUPPORT** independence – **LEARN** what works

THE DEMONSTRATOR WILL TEST WHETHER IT CAN

- Reduce avoidable repetition, failed hand-offs and professional workload.
- Improve recovery, independence and participation for carers and carees.
- Release carers' capacity to care - and to work where possible.
- Use public money more effectively, with outcomes measured before claims are made.
- Produce a practical model that can inform national implementation.

EVIDENCE TO COLLECT

EXPERIENCE
What carers and carees say changes.

CAPACITY
Time saved or released across services.

OUTCOMES
Recovery, independence and participation.

We are not asking the Commission to accept the hypothesis.

We are asking the Commission to help us test it.

Pilot first. Platform second. Evidence before scale.

THE CASEY ASK

Enable the test.
Convene the partners.
Review the evidence.

A PROPOSED OVERSIGHT GROUP

CASEY COMMISSION	convene + independently review
NHS ENGLAND	digital, data + NHS App
SWL ICB	local NHS governance
ADULT SOCIAL CARE	Richmond + Wandsworth
HOUSING	council + provider partners
DWP + VCSE	benefits, work + community
CARERS + CAREES	co-design + consent
INDEPENDENT REVIEW	measure + publish results

WHAT WE ARE ASKING CASEY TO DO

1. Help convene the partnership and enable the six-month test.
2. Protect consent, confidentiality, separate rights and clear ownership.
3. Support an independent review and publication of the result.

We are not asking Casey to accept the hypothesis.
We are asking it to help us test it.

Pilot first. Platform second. Evidence before scale.

CONNECT THE DECISIONS. IMPROVE THE OUTCOMES.